

Book Reviews

From Under the Cloud at Seven Steeples. 1878–1885.

The Peculiarly Saddened Life of Anna Agnew at the Indiana Hospital for the Insane. By Lucy Jane King. Guild Press/Emmis Publishing, LP, Zionsville, IN, 2002; 189 pp., \$24.95 (hardbound).

Cicero taught us that history is the teacher of life (“*Historia magistra vitae*.”). Historical accounts of various episodes, institutions, methods in each discipline, including psychiatry, are important, illuminating, and educational. Hopefully, yet not necessarily, they help us avoid repetition of past mistakes, and help us understand the evolution of the discipline, treatments, laws, and other issues. The addition of personal accounts or extensive excerpts from personal accounts makes historical treatises even more attractive, entertaining, and readable.

Lucy Jane King, Professor of Psychiatry Emeritus at Indiana University with a major interest in the history of psychiatry, brings us such a combination of historical treatise and excerpts of a personal account. Her book, *From Under the Cloud at Seven Steeples*, describes the life in a nineteenth-century asylum, drawing from a fascinating personal account of Anna Agnew (1). Anna Agnew spent several years (1878–1885) at the Indiana Hospital for the Insane (later Central State Hospital, closed in 1994), called Seven Steeples (the hospital actually had eight steeples, but, according to a legend, only seven steeples could be seen from any given point along the road outside the grounds). She was hospitalized and “treated” for bipolar disorder. Dr King uses Agnew’s account of the hospitalization and treatment as a vehicle into contemplations and commentaries about the history of psychiatric diagnosis and treatments, the relationship between bipolar disorder and creativity, stigmatization of psychiatry and psychiatric patients, and women’s issues.

Part one of the book, which consists of eight chapters, intertwines the excerpts from Anna Agnew’s book with the author’s commentaries, discussion of diagnosis, historical and genealogical information, and illustrative accounts about Anna Agnew’s family and her treating physicians. Interesting is Anna Agnew’s account of her major depression, mood swings, and “periods of excitement” in chapter three. At times her guilt obviously reached a delusional stage. She also discusses her suicidal thoughts quite eloquently.

Another chapter contains a detailed account of the daily life in the nineteenth-century asylum (“the institution was a village itself”), with a production of serviceable garments and linens for inmates, an early version of occupational therapy. Interesting is also the discussion of the late nineteenth-century psychiatric armamentarium: salicylates, morphine, bromide salts, and chloral hydrate, i.e., mostly sedatives. In addition, Dr King educates the reader about the cyclical nature of the reform of psychiatric institutions and policies. The final chapter of this part discusses the fate of Anna Agnew after the asylum: recovery in the absence of any effective treatments, divorce, return of her illness, rehospitization, and death in another psychiatric hospital years later.

The second part of the book contains two chapters, one on the development of psychiatry during the last century and the other on the significance of Anna Agnew’s account for the twenty-first century psychiatry. The book is accompanied by explanatory notes and a set of interesting photographs, most of them from the collection of Indiana Medical History Museum.

This is an interesting little book. Although it is written for a wider, lay audience, it is interesting reading for psychiatrists, too. It provides an illustrative, fascinating insight into nineteenth-century psychiatry, its abuses, treatments, and politics, and even its positive, human side. It also discusses the implication of the nineteenth century for the mental health care in the twenty-first century. It could be entertaining bedtime or beach reading for history buffs and those interested in the evolution of our discipline.

REFERENCE

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Emotionally Focused Couple Therapy With Trauma Survivors. Strengthening Attachment Bonds.

By Susan M. Johnson. The Guilford Press, New York, 2002; 228 pp., \$30 (hardbound).

The impact of psychological trauma on an individual and development of psychopathology in the traumatized individual has been appreciated by mental health workers. Accordingly, numerous pharmacotherapies and psychotherapies focusing on the treatment of traumatized individual(s) have been studied and found useful in the treatment of persons with posttraumatic stress disorder (PTSD). However, as the proponents of the attachment theory suggest, trauma also has a tremendous impact on a person's capacity to trust and feel safe in an intimate relationship, and also has a serious impact on the traumatized person's partner.

Susan Johnson, one of the developers of the emotionally focused couple therapy, proposes that emotionally focused couple therapy with traumatized individuals and their partners can play a very important role in the comprehensive treatment of trauma survivors. She suggests that "we all know it is better not to be alone in the dark and that connection with other makes us stronger." Dr Johnson also takes the position that if a person's connection with significant others is not part of the coping and healing, then it becomes part of a problem, obstacle for recovery, and even a source of retraumatization. Thus, Dr Johnson wrote a book, which "offers a general approach to therapy with traumatized couples, for whom affect regulation and safe emotional engagement are key issues."

The book is divided into two parts. Part I, "Trauma in context: Healing in attachment relationships," consists of five chapters. It provides rather general information about couple therapy from the attachment theory point of view. Chapter one, "Healing connections: Expanding the role of couple therapy," is a general introduction to couples therapy from the attachment theory point of view. It also outlines the scope and intent of the book. Chapter two, "Trauma and its aftermath," starts with the discussion of PTSD and its nature and symptomatology. The chapter continues with review of the trauma trap ("in which one level of impairment prevents self-regulatory healing process from occurring on other level"), retraumatization, coping with trauma, healing from trauma, treatment of trauma, individual therapy for trauma, and recovery from trauma. Recovery from trauma, according to the author, consists of three stages: stabilization, working through the trauma and building self and relational capacities, and consolidation and integration.

Chapter three, "Attachment and trauma," provides a solid introduction to the attachment theory,

starting with the 10 central tenets of this theory. The chapter further deals with topics such as attachment theory and the quality of adult love relationships, attachment bonds and healing, attachment relationship and trauma, attachment styles and trauma survivors, and couple therapy with traumatized patients. The author states that from the attachment perspective, negative cycles of blame/pursue and withdraw/distance are so harmful because they destroy the safety and responsiveness necessary for sustained emotional engagement and secure bonding. She also suggests that anxious, preoccupied partners are always afraid of losing attachment figures, and so they tend to be reactive to affective cues and to amplify negative emotions by attending to them excessively. They tend to drive others away, confirming all the anxious partner's fears. On the other hand, in the individuals with avoidant styles emotion is expressed in somatization, hostility, and avoidance, such as obsession with instrumental tasks. This is a very useful and crucial chapter for the understanding of the whole book.

Chapter four, "Assessment," starts with several short clinical vignettes (which are presented in full length later), and then gets into issues such as key dimensions of first sessions (level of trauma, personal/relationship trauma, level of personal integration of trauma, and level of integration of trauma into the relationship), and the question whether trauma is part of the relationship problem. The author then discusses safety nets and the importance of therapeutic alliance ("the best safety net is a positive therapeutic alliance"), assessment instruments (e.g., Dyadic Adjustment Scale), the process of early sessions, and the goals of individual sessions. Some practical hints in this chapter: the best evaluation is a good interview (not the questionnaires), and one of the tasks during the initial sessions is assessing whether the intervention is presently viable. Chapter five, "Interventions," reviews special issues of the beginning therapy and expands on the three stages of recovery outlined in chapter two (stabilization, restructuring the bonds between partners, integration).

Part II, "Clinical realities: Couple therapy with traumatized couples," contains six chapters. The first five chapters of this part are mostly cases from clinical practice, expanding on certain points made in the first part. Each of these chapters starts with a brief introduction and then reviews the therapy process, going through the various stages (stabilization, restructuring). The author provides specific examples of dialogues between the couple and the therapist. Chapters deal with specific issues, such as "defeating an anxiety

disorder and marital distress,” the trauma of physical illness and its impact on the relationship, couple therapy with combat veterans, and attachment injuries in a close relationship. Interestingly, as the author suggests, the actual incident that precipitates an attachment injury is not necessarily the primary causal factor in a couple’s marital distress. The last chapter, “Therapists who deal with dragons,” provides some advice to therapists on how to deal with the emotional strain of couple therapy with trauma survivors. The author discusses the role of the therapist, self-care for the therapist, and the rewards of working with traumatized couples. Dr Johnson suggests that being aware of one’s limits, avoiding being swept by countertransference, consultation with colleagues, and therapist self-disclosure are the main parts of the therapist’s self-care. I do not believe that everybody would agree with the self-disclosure part of these measures.

This is an interesting and useful book. One may or may not like all of its parts and suggestions. However, it provides a fairly coherent message, which is not always a feature of books on various therapies. It is a very good example of the use of the attachment theory in clinical practice. It explains well the rationale for the emotionally focused couple therapy with trauma survivors. It provides a wealth of clinical examples of this therapy. It is fairly practical and well written. Couple (formerly marital) therapists, family therapists, trauma specialists, and teachers of psychotherapy may find it useful. This book may also be useful for teaching couple therapy and the clinical application of, at times boring, yet important attachment theory to residents.

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A Community Reinforcement Approach to Addiction Treatment. Edited by Robert J. Meyers and William R. Miller. Cambridge University Press, Cambridge, United Kingdom, 2001; 188 pp., \$74.95 (hardbound).

The quest for successful alcohol and other addiction treatment has been a long, difficult, and mostly unsatisfactory journey for many clinicians and researchers. Many approaches have been enthusiastically endorsed and later discarded. Only a few concepts in this area, such as Alcoholics Anonymous and permanent abstinence, withstood the test of time. Most addiction treatments have been based

in behavioral theories and approaches. However, as the authors of the monograph on community reinforcement approach to addiction treatment point out, the American-style disease models have looked almost exclusively within the individual for the cause of alcoholism and drug addiction and overlooked the important environmental determinants of substance use disorders. This little monograph from the International Research Monographs in the Addiction series focuses on a behaviorally based treatment of addictions—the Community Reinforcement Approach (CRA)—which takes the environment into account. The community reinforcement approach to treating alcohol and other drug addictions “is designed to make changes in the patient’s daily environment, to reduce substance abuse and promote a healthier lifestyle.”

This monograph reviews CRA’s basic tenets and focuses mostly on the research of CRA and the evidence of its effectiveness. The book, edited by Drs Meyers and Miller, is divided into nine chapters authored by CRA researchers/experts/proponents from New Mexico, Illinois, and Vermont.

Chapter one, “Developing the Community Reinforcement Approach,” briefly describes the establishment and development of the original CRA program by Nathan Azrin and his group at Anna State Hospital in Illinois. CRA was the brain child of Azrin’s doctoral student George Hunt. CRA is based on learning theory and in particular on the operant approach. It stresses the interaction between a person’s behavior and the environment. It also suggests that the etiology of alcohol problems is influenced by patterns of positive and negative reinforcements, which could possibly maintain drinking indefinitely. Hunt and Azrin also proposed that drinking could be reduced if reinforcers of *not* drinking, such as a better interpersonal relationship or satisfying employment, were maximized, frequent, and contingent upon not drinking alcohol. Chapter two, “Practice and Promise: The Azrin Studies,” reviews the initial test studies evaluating the effectiveness of CRA by Azrin’s group, starting with the original 1973 study. It describes the major components of CRA during the initial trials: job-finding procedures, behavioral marital therapy, social and leisure counseling, a Social Club, and home visits. Newer components, added in later studies, included disulfiram prescription, disulfiram assurance procedure, problem-solving, buddy system, early warning/mood monitoring, motivational counseling, sobriety sampling, muscle relaxation training, and others. The original Azrin studies (1973, 1976, 1982) demonstrated

the efficacy of CRA, especially in areas such as decreased percentage time drinking, time unemployed, time away from the family, and time institutionalized.

Chapter three, "The treatment," provides a very detailed outline of the previously mentioned components of the CRA treatment program and a functional analysis of the CRA. Samples of assessment forms are provided (e.g., Initial Assessment, Happiness Scale, Relationship Happiness Scale, Daily Reminder To Be Nice—some of them with examples of a completed form), and their use in treatment planning is discussed. The authors introduce a unique feature of the CRA functional analysis: both drinking (or drug-using) and pleasurable, nonproblematic behaviors are examined routinely with the goal of decreasing the former and increasing the latter. They suggest that it is important to introduce pleasant activities that can compete with and replace drinking behaviors. They also propose a gentler way of introducing abstinence—so-called sobriety sampling. Also included is a solid description of CRA relationship therapy, social/recreational counseling, and CRA's relapse prevention. At the end, the chapter's authors warn the reader that, as with all behavioral programs, CRA is only as good as the therapists who administer it. This is a very useful, practical, clinically oriented chapter. It would be the most important one for those interested in learning and starting a CRA program.

The next four chapters report the results of research studies on CRA. Chapter four, "A Comparison of CRA and Traditional Approaches," is basically a detailed methodology section of a study comparing CRA to traditional treatment alone and to traditional treatment plus disulfiram. Chapter five, "Community Reinforcement and Traditional Approaches: Finding of a Controlled Trial," presents the results of this study. The main study findings are a bit complex. The tested treatments appeared to be equally acceptable. CRA was more successful in suppressing the frequency of drinking than was traditional treatment without disulfiram monitoring. However, in the presence of disulfiram monitoring, CRA and traditional treatment did not differ in efficacy. Adding disulfiram monitoring did not improve CRA's efficacy, but improved the traditional treatment efficacy. However, the outcome clearly depended on the outcome measure used. When the traditional outcome standard of continuous abstinence was used, traditional treatment with disulfiram fared better than CRA with disulfiram. When frequency of drinking was used as an outcome measure, the results were the opposite (authors:

"this study was no simple horse race"). A very useful part of this chapter is the discussion of the pitfalls of CRA research studies.

Chapter six, "CRA with the Homeless," describes a study of CRA in alcohol-dependent homeless individuals. CRA outperformed the standard treatment. Chapter seven, "CRA and Treatment of Cocaine and Opioid Dependence," reviews studies of CRA in cocaine and in opioid dependence. Very interesting is the discussion of voucher-based initiatives (patients earn vouchers for cocaine-negative urine toxicology test). The presented studies support the efficacy of CRA treatments for cocaine and opioid dependence. Chapter eight, "Community Reinforcement and Family Training (CRAFT)," emphasizes that family members can play a powerful role in helping to engage a resistant loved one in therapy. CRAFT is a program for family members and friends who hope to maintain their relationship with the resistant substance user, but who want it to change in a positive direction. It teaches the concerned significant others how to use positive reinforcement, negative consequences, and how to reward themselves.

The last chapter, "Summary and Reflections," is an excellent summary of lessons learned: CRA works, CRA is teachable, CRA helps in relapse prevention, involvement of significant others makes a difference, positive reinforcement works, the response to CRA is a broad spectrum one, CRA need not be expensive, and disulfiram is not necessary. An excellent point made by the authors is that prognosis, like motivation, is not merely a matter of patient characteristics. It also occurs in the context of available treatments. The authors contemplate why CRA has not been used more broadly. One of the reasons is that, until very recently, there were no manuals to guide therapists in the application of CRA (one is available now (1)). The final part of this chapter outlines some future directions for CRA.

This is an informative monograph about a lesser known, yet attractive, effective, and interesting addiction treatment approach. The book is well conceptualized and organized. However, its clinical usefulness is probably somewhat limited. Although the clinically or instructionally oriented chapters (namely chapters 3, 5, and 9) are useful, one would assume that the referenced manual of CRA (1) is probably more instructive and practical for a busy clinician. Most of the chapters present results of research studies of CRA. Thus, this book would probably be more useful for addiction researchers and teachers, or those who would like to justify starting a CRA treatment program. Last but not

the least, the book is not exactly cheap, considering its size.

REFERENCE

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Seeking Safety: A Treatment Manual for PTSD and Substance Abuse. By Lisa M. Najavits. Guilford Press, 2002; 401 pp., \$35.00 (paperback).

This book is a treatment manual developed to address the needs of patients who concurrently experience two of society's more debilitating and costly disorders: substance abuse and posttraumatic stress. The author is a practicing clinical psychologist who has researched and published extensively on substance abuse, traumatic stress, and treatment efficacy and outcome. In this manual she offers an approach to treatment grounded in empirically validated cognitive-behavioral therapy (CBT) that combines aspects of traditional substance abuse and posttraumatic stress disorder (PTSD) treatments and also attempts to fill in the gaps between them. The author bases her approach on five central principles: establishing safety; integrating the treatment of PTSD and substance abuse; a focus on (loss of) ideals and on potential rather than on symptoms or pathology; cognitive, behavioral, interpersonal, and case management content areas; and attention to therapist processes. The "Seeking Safety" of the title refers to the development of coping strategies, that is, learning to decrease those behaviors that place one in danger (such as exposure to HIV or suicidal attempts) and replacing them with behaviors more consistent with self-care and good health. The structured format of this treatment manual provides the therapist with tools for teaching patients how to develop those self-care behaviors.

The manual is divided into two main sections: two chapters which introduce important topics, provide the basis for the treatment, and describe how to conduct treatment sessions, and 25 chapters of "treatment topics," each of which addresses session content on how to develop safety behaviors. A brief Preface

places the manual in the context of the author's personal life and her professional training and experiences. It also mentions both the advantages and limitations of conducting psychotherapy via a structured manual format.

Chapter One cites a large number of sound clinical and research studies to lay out the relationships between PTSD and substance abuse, and gives the reader a glimpse of the extent of the psychological damage and societal costs of these disorders. This chapter presents an excellent, although brief, summary of the difficulties inherent in trying to provide treatment to patients who are much more complicated than a single diagnosis or condition. Of particular importance, it highlights a major deficiency in the training of those professionals currently working with PTSD or substance abuse patients: most of us were not taught how to treat both at the same time. As the author points out, however, the evidence of the past decade strongly suggests that an integrated model will be more successful and cost effective than a sequential or parallel approach.

Chapter One also provides a brief description of the core elements of CBT, and makes the case for its use in teaching this patient population how to develop safety behaviors. In Chapter Two, the author does a masterful job of balancing recommendations for adherence to the manual as written and encouraging clinicians to adapt its tools as they see fit. She does not talk down to seasoned clinicians when reviewing elementary concepts, instead handling basics in a manner that comes across as a "booster session." At the same time, this is not a training manual for a novice therapist or a clinician with no background in CBT. While the session content sections seem sufficiently detailed to allow an experienced clinician to "jump in," prior training in CBT and some experience with PTSD and substance abuse patients would seem to be a must. (The author notes that additional training materials are available in the form of a published article and videos.)

Besides attending to PTSD and substance abuse problems simultaneously, this treatment manual fills other gaps as well. One of its major strengths is its balanced attention to research and to clinical needs. Most textbooks and research articles tell us how one treatment compares with another in terms of outcomes, but do not give us much help with the specifics of providing those treatments to the patients we actually see in our offices. Practice guidelines may be outlined, but often our patients do not really match the inclusion/exclusion criteria of scientific studies, do not

come in reliably for the recommended number of sessions, or just are not ready or able to engage in intensive treatment. Many treatment manuals, on the other hand, are all-inclusive and easy to implement, but seem to be written with little if any regard to what the empirical literature tells us is worthwhile. There are but a handful of treatment materials currently available that meet rigorous scientific standards for providing treatment for anxiety, PTSD, or other problems, and this manual is the first I have seen to address PTSD and substance abuse. It gives us a way of getting our patients to the “state of the art” treatments (e.g., exposure therapy for trauma) that appears to be better than any way we have had before now.

Another strong point of this manual is the way the author integrates often-neglected existential issues into her treatment approach. While traditional substance abuse treatments such as Alcoholic Anonymous have for years talked about the necessity of addressing spiritual and philosophical needs, most psychological- and biological-based therapies for other mental health problems have not considered this to be as important as changing behavior or biochemistry. However, trauma patients typically insist on a discussion of responsibility, shame, guilt, and hopelessness, and the manual gives those of us who may be uncomfortable with these topics an enlightening way to serve our patients’ needs.

The manual is well-written, satisfying, and easy to read; it provides a rational approach to an area sorely in need of empirically based treatments. It challenges and inspires established clinicians to think about this very difficult patient population in a different way, and provides structure for therapists just starting their work. Future research will determine if this treatment approach will continue to be a productive one; in the meantime the manual gives us a way to feel we are trying our best.

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How Brains Make up Their Minds. by Walter J. Freeman. Columbia University Press, New York, March 2000; ISBN: 0-231-12008-7; 171 pp., \$24.95, (hardcover).

This deceptively small book is the sixth volume in Columbia’s series, “Maps of the Mind.” The series

general editor is Steven Roose. The author of this volume, Dr Freeman, is a professor in the Graduate School of the University of California, Berkeley, and has taught brain science for over 40 years. He had several hundred articles and three books to his credit prior to the publication of this one. *How Brains Make up Their Minds* consists of seven chapters, an acknowledgement, bibliography, and index. In brief, this book explicates Dr Freeman’s theory of how awareness, consciousness, and choice—the major determinants of the “mind” in his definition—arise from the structure and physiology of the central nervous system. In the first chapter, he says that to do so, he must satisfy three conditions. First, explain the brain mechanisms through which neurons construct the options for choice. Second, explain what is happening in the organization at the moment of choice. And finally, explain in neural terminology the nature and role of awareness and how it is linked to consciousness. If this sounds like quite a task, I would agree. Chapters one through five work toward the first two conditions, while the last two discuss the third. Therefore, the initial chapters rely much more upon data with smatterings of philosophical discussion and the final two are just the opposite.

Dr Freeman believes the hippocampus is the seat of meaning in the brain. He goes on to describe 10 building blocks of the “dynamics of intentionality,” including how neurons form populations which are more important to choice than individual nerve cells, how the electrical oscillation activity of these cells provides a biological background for choice, and how limit cycles produce a steady state in this oscillation. He feels that a background of chaotic activity in the neuron populations has an overlying amplitude modulation (AM). Gain in the AM is input dependent, and AM patterns are dependent on context, history, and significance of sensory input and experience. He notes that all the brain receives in the way of sensory input is what the sensory end organs have constructed, and finds that multisensory convergence into the entorhinal cortex allows for global integration in the limbic area. This creates a context for space–time orientation to incoming data. Connections from the limbic or emotional portion of the brain radiate to the motor cortex, which is the basis for action generated by choice. “Patches” of neurons are either activated or inhibited to form a global pattern that varies in the face of seemingly identical stimuli because it is the brain as a whole that generates consciousness in Dr Freeman’s explanation. While the internal experience of the brain is isolated, the sensory and motor

connections allow for social interaction, which is seen as the true function of the human organism. We must “unlearn” old patterns before we can learn new ones, as the mind performs novel tasks throughout life.

Dr Freeman has already condensed his work into seven chapters and 18 figures to produce this book, and so the one paragraph summary above can only impart the building-block approach taken by the author. This book covers a wide range of scientific literature and philosophical debate about the brain. It ranges from discussion of cave paintings to rave dances. I would recommend it particularly for educators looking for a global explanation of how the brain may function in order to update lectures and seminars for students in the medical and psychological sciences. For anyone who wishes to expand his or her appreciation of how one can assemble the varied data we currently have regarding the brain, this book provides a well-researched and interesting approach. I believe the author has succeeded in creating a theory of the mind which bridges the gaps between the data provided by neuroscience, physics, neurology, psychiatry, philosophy, sociology, and psychology.

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A Guide to Treatments That Work, 2nd ed. Edited by Peter Nathan and Jack M. Gorman. Oxford University Press, 2002; 681 pp., \$85.00.

This is a fine revision of a well-received textbook that covers a timely topic in psychiatry, evidence-based treatment. In an era when psychopharmacology and polypharmacy dominate clinical psychiatry practice, this text covers in depth, the level of evidence for these treatments. Being true to its title, the text also addresses the evidence for psychological treatments of the mental disorders in almost equal measure.

Regrettably, only a small part of common psychiatric practice is studied and known to be effective. For example, treatment of common psychiatric conditions in the medically ill and treatment of patients with multiple psychotropics is nearly unstudied. Over 50 authors, many known for their work in the field, contribute to 26 chapters covering approximately 20 mental disorders.

The book focuses on levels of evidence, giving greatest weight to the randomized controlled trials, and sometimes mentioning open treatments when less

informative data, such as cohort studies or case series, are available. The Preface gives a lucid discussion about levels of evidence from research, even though the chapters infrequently follow-up on the hierarchy they outline.

The chapters vary in style, though they are generally brief and focused. Like journal articles, each chapter begins with a summary of the chapter that follows. Some chapters, especially the chapter *Pharmacologic Treatments for Unipolar Depression*, gives background and the nuances for understanding the meanings of results, the limitations of clinical trials, and the limitations of the evidence. Other chapters dryly run through the randomized trials and report on the results of the trials, noting the measures on which drug was superior to placebo, but often mentioning neither the clinical significance nor the effect sizes (difference between the outcome measure of the drug and placebo divided by the standard deviation) found in the trial. Unfortunately in the pharmacologic treatment of many disease states (i.e. alcoholism and treatment-resistant schizophrenia), although a drug may be superior to placebo, the difference between the drug and the placebo may be modest. Additionally, in some illness states the change in symptoms from the beginning to the end of treatment may be modest or clinically unimpressive. These facts are sometimes omitted in reporting on treatment results.

Like the drug therapy studies, the psychosocial therapy treatments are well written and focus on studies and results. The psychosocial treatment chapters interestingly often give explanatory paragraphs about the illness states, as well as rationale and methods of treatment, evidently predicting that their audience will be readers who are more informed about pharmacologic than about psychological treatments. They detail the available controlled studies or meta-analysis reports and discuss which treatments are effective. These chapters, more than the pharmacology chapters, discuss effect size and limits of the research findings.

The chapters are uniformly well written and stick to evidence. Each chapter also includes tables, sometimes to excessive length (up to 25 pages), for the studies that have examined the treatment at issue.

The book begins with an excellent 10-page table that summarizes psychological and pharmacologic treatments and the standards of proof (randomized trials or lesser studies). It serves as a very good summary and table of contents, directing readers to the appropriate chapter pages for further reading. Curiously, the topic of dementia treatment, which

could most benefit from a summary of the studies because of its burden of tables, is absent in this section.

This book is up to date, with many references published in 1999 and 2000. It should be a welcome addition for any clinician and would serve as a fine reference text or source for teaching residents in psychiatry. Although the book examines clinical studies, which are arguably distinct from clinical practice, it is clinically relevant and digestible. For the person wanting to know the extent of our knowledge about the state of treatment, this is an excellent book.

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Beyond Reason: Art and Psychosis. Works from the Prinzhorn Collection By Bettina Brand-Claussen, Inge Jadi, and Caroline Douglas. University of California Press, Berkeley, and Hayward Galleries, London, 1998; 195 pp. \$35 (paperback).

Fascination with the artistic productions of individuals with severe mental illness has existed for millennia. They have been considered to provide a window into the origins of creativity as well as a view into the patients' thought processes and illnesses.

This catalogue accompanied an exhibit of such works at the Hayward Galleries in London in 1996. It represents a selection from the collection of artistic works by over 450 patients in mental asylums assembled by the German psychiatrist and art historian Hans Prinzhorn (1886–1933) in the early part of the twentieth century. The impetus for this collection was his belief that the art of mental patients was useful in providing an insight into pure, primitive origins of art unencumbered by external, outside influence as well as providing a therapeutic outlet for such patients. The collection, eventually consisting of more than 5000 pieces, while not originated by Prinzhorn, was popularized by him in the book *Artistry of the Mentally Ill* published in 1922, which espoused these ideas. It eventually attracted the attention of many avant-garde artists of the time and was used to deprecate their work as “degenerative” by the Nazis. The collection fell into disfavor early in World War II but was rediscovered after the War by the Art Brut or Outsider art school and remains an inspiration for modern artists. The present collection was housed in the Heidelberg

Clinic Hospital where it was rediscovered in the early 1970s. It subsequently was restored and catalogued, and attempts were made to provide historical background of the artists. In the mid-1990s, it began to be exhibited throughout Europe and the United States until permanently housed in a museum at the University of Heidelberg.

The book contains representations of over 200 works—collages, prints, doodles, drawings, cartoons, sculptors, post cards—in color and black and white. It is accompanied by three essays by Bettina Brand-Claussen, assistant curator of the collection, Inge Jadi, curator at the Department of Psychiatry of Heidelberg University, and Caroline Douglas, exhibition officer at the British Council. These provide an insight into the history and psychological, ethical, and artistic place of these works.

Brand-Claussen provides information about the patients, some of whom appear to have been talented artists prior to their confinement, and the political use made of these works, which continues to cause controversy. Depictions of sketches by the patients together with those of so-called degenerative artists, which was used by the Nazis, demonstrates this.

Jadi's essay struggles with the place these artists' productions have as art versus mere outpourings of the severely mentally ill. She also raises ethical concerns about the use of such works and their exploitation for the benefit of others and not the patients. Dr Jadi displays a sensitivity to the needs of these patients as represented in their works appropriate for a psychiatrist.

Douglas provides an excellent history of the development of the collection in terms of merging art considered to be psychopathologic into the area of true art. She also discusses reflections of social influences on such art particularly as they relate to the emerging and changing role of women in the early twentieth century. Her comments about the features of some of the works provide interesting interpretations into the minds of these patient artists, reflecting their pain and perhaps the reasonableness of their purpose.

The works are accompanied by brief sketches of what is known about the artists, which unfortunately seems to be minimal. Their diagnoses, when indicated, reflect the limited diagnoses of the time, e.g., imbecility, various forms of schizophrenia (dementia praecox), senility, or paranoia. I suspect that many would be considered to have suffered from bipolar spectrum disorders today. Unfortunately, most of the patients died or were exterminated as a result of Nazi ideology.

I am neither an artist nor knowledgeable about art or art criticism. However, viewing the various pieces in the catalogue proved to be overwhelming and stimulating. They varied from highly technical, machine-like, detailed offerings to repetitive themes, cartoons, incomprehensible etchings to very artistic productions of obviously skilled artisans. My first inclination was to attempt to place a diagnosis on the artists. However, that not only proved nonproductive but also detracted from the works. The pictures of several artists are collected in detail. At times, I found myself feeling as if I were actually wandering through the gallery viewing each piece as art, irrespective of the patient's background. Of course, as a psychiatrist, I eventually began to associate some of them with their psychotic, manic, or depressive states of mind—a reflection of their illnesses, their feelings, and struggles to deal with them and their perceptions of their surroundings.

The catalogue brings into perspective the association between artistic creativity and the psyche of mental patients into providing the origins of imagination. Those who struggle to advance their own artistic abilities look to these individuals and their chaotic, undisciplined minds as a source of enlightenment, inspiration, and expression. While these patients did not think of themselves as artists, their works certainly stimulate, educate, and arouse as works of art even if they originated in minds frequently devoid of reality and seeking relief from torment.

I am uncertain of the place of this volume in my book collection. It is certainly not a coffee table item nor appropriate for placement in my office waiting area. Perhaps, I shall keep it close by for perusing at leisurely times when I can allow my mind to wander and begin to feel a sense of closeness to the artists. It certainly provides a fascinating view of the association between psychiatry and art. For those with such interest and inclination, this book provides an excellent companion piece.

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Theater of Disorder: Patients, Doctors, and the Construction of Illness By Brant Wenegrat. Oxford University Press, New York 2001; 292 pp., \$35.00

A reviewer receives all sorts of books to read. Some are just so-so, some terrific. Then, every once

in a while, along comes one you wish you had written yourself. Such a volume is *Theater of Disorder*, by Brant Wenegrat, an Associate Professor of Psychiatry at Stanford. It sets out to “apply a theatrical model of social relations” to disorders such as hysteria, chronic fatigue syndrome, and dissociative identity disorder, making ports of call at other, less well-known conditions with roots deep in demonology—latah, negi negi, dancing mania, witchcraft, lovesickness, and demon possession. Along the way, he demolishes hypnosis as a tool for memory recovery (though he acknowledges its value in recovery from psychogenic amnesia).

The argument in brief: patients and clinicians unconsciously work together (I will not, nor does Wenegrat, say “conspire”) to produce a simulacrum of disease that often works to the eventual destruction of the patient. In his view, though clinicians may give powerful cues, it is largely the patients' perceived needs that provide the impetus to adopt the illness role.

Wenegrat sets the scene with reviews of historical patients. Along with many of the lesser-known, these include Freud's Dora and Charcot's Bertha (though he should have noted that the latter's hysteria patients did not devise their epileptiform *Grand Hysterie* all by themselves, but by imitating the genuine epileptic patients with whom they were housed and for whom they helped provide care). He also provides details of the basic research, history, and anthropology that lie behind these representations of illness. He describes the features of disorders that, within Western culture, lend themselves to the illness role: no objective physical or physiological findings; narrow distribution (just a few practitioners or particular societies); fad syndromes with mushrooming incidence; some illnesses that are “legitimate” for some patients providing illness roles for others (e.g., Gulf War and chronic fatigue syndrome).

I would like to debate a couple of points. Wenegrat seems to imply that patients and clinicians alike are out to deliberately deceive one another. After decades of study, I have come to believe quite the opposite—that most patients with somatization disorder and similar illness have no such intent at all. Patient and clinician are caught up in a script, all right, but it is as though one penned the words, one the stage directions, without realizing the contributions of a coauthor.

The statement (page 84) that “grand hysteria lacks direct descendants . . .” derives its truth from the symptoms themselves, which, to be sure, disappeared

abruptly at Charcot's death in 1893. However, the underlying condition responsible, call it what you will—Wenegrat explores the various terms (hysteria, Briquet's syndrome, somatization disorder)—is still with us and probably will endure for millenia to come. It is, to coin yet another term, a disorder that is not iatrogenic but *iatroplastic*, not caused by doctors but shaped by them. Its sufferers require care so intensely that they will conform to whatever expectations they perceive their caregivers to hold. In this sense, I would argue for a clinician's role that is even stronger, though perhaps less consciously undertaken, than the one put forward by Wenegrat.

Nonetheless, in these pages there is much to admire and agree with. I particularly like Wenegrat's

annotation, which lists and also discusses references in 40 pages at the end of the book. The sweep of this work suggests that illness roles are likely to evolve, from railway spine to grand hysteria to shellshock to . . . who knows what lies beyond, once we have vanquished dissociative identity disorder, multiple chemical sensitivity, and alien abduction syndrome? Will we, as Wenegrat suggests, continue to invent illness roles? This well-written work is one of devotion to true scholarship; I recommend it to everyone who cares about the history and future of psychiatry and the welfare of mental health patients.

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